# Fax cover sheet

**Seaside Community Medical Center (fictional)**

Case Management Department

**FAX TRANSMITTAL**

To: Illumination Health + Home - Recuperative Care Intake
Fax: 555-0199
From: Alicia Romero, MSW
Phone: 555-0195
Date: 09/05/2026 09:50
Pages (including cover): 11
Re: Recuperative care referral
Urgent [X] For Review [X] Please Reply [X]

**Message**

24M L forearm lac w/ cellulitis, on PO abx. Ambulatory, independent.
Ready for discharge today.

**CONFIDENTIALITY NOTICE**

This facsimile contains confidential health information. If received in error, notify sender and destroy.

-- FICTIONAL DEMO RECORD - not a real patient --

# ILLUMINATION Recuperative Care Referral Form

HEALTH + HOME

To refer a patient, please complete and submit this form with supporting documents via email to RECUP@ifhomeless.org or FAX to (888) 382-9551. For referral questions, please contact an Illumination Referral Coordinator at (888) 505-0855.

## Referring Party

Referring Hospital: Seaside Community Medical Center (fictional) Dept/Floor: 5 Medical
SW/CM/RN: Alicia Romero, MSW Phone/Pager #: 555-0195
Email: aromero@seaside.example Nursing Station #: 555-0196
Authorized by: Dr. F. Nakamura Phone #:

## Patient Information

Patient Name: Kevin Doe MRN: SC-5072218 Date of Birth (dd/mm/yyy): 02/17/2002
Gender: ☒ Male ☐ Female ☐ Transgender ☐ Other Ethnicity: Hispanic/Latino
Last 4 # of Social Security: xxx-xx- 0000 Insurance: Orange Coast Health Plan (fictional) Medi-Cal Preferred Language: English
Housing Situation: ☒ Currently Unhoused ☒ 4+ episodes of homelessness in the past 3 years

## Hospital Admission Details

Chief Complaint Admitting Diagnosis:
L forearm laceration with cellulitis
Admit Date: 09/01/2026 Any Wounds? ☒ Yes ☐ No
If yes, explain the number, location, size and stage:
1 - L dorsal forearm laceration 6 cm, sutured
Independent with Wound Care? ☒ Yes ☐ No If no, will Home Health be ordered? ☐ Yes ☒ No
Was PPD/TB Test or Chest X-Ray Performed? ☒ Yes ☐ No If yes, Date: 09/01/2026 Results: CXR neg

## History of Health & Living Situation

Does the patient require or have... (Please circle Y or N and provide explanation accordingly.)

| | | | |
|---|---|---|---|
| O2 | Yes* X No | Active Infection | X Yes* No |
| *Explain | | *Explain | cellulitis - PO abx |
| Self Administer All Meds | X Yes No* | Ambulatory | X Yes No |
| Continent of Bowel & Bladder | X Yes No | Assistive Device | Yes* X No |
| Colostomy/Ileostomy | Yes X No | *Explain | |
| Foley Catheter | Yes X No | Hospice | Yes* X No |
| Diabetes | Yes X No | *Explain | |
| Insulin | Yes X No | Car | Yes X No |
| Diabetes related Oral Meds | Yes X No | Service Animal | Yes X No |

## Behavioral and Mental Health Status

Does the patient have any limitations, behavioral or mental health challenges? X No Yes

☐ Mental Health DX If checked, what type?
☐ Non-compliant ☐ Forgetful
☐ Cognitive Impairment ☐ Registered Sex Offender
☐ Other Explain None

## Length of Stay Authorized in Recupertive Care: 14 Days

### ILLUMINATION STAFF ONLY

Date Received: Reviewed by:
Approved: ☐ Yes* ☐ No** *Date: *Authorization #: **Reason for Denial:
Admission Date: Admission Time: Exit Date:
Extension Request: ☐ Yes ☐ No If YES, Additional Approval Date: by Total LOS

# CalAIM Community Supports authorization request

**CalAIM COMMUNITY SUPPORTS - AUTHORIZATION REQUEST**

Service: Recuperative Care (Medical Respite)

Health Plan: Orange Coast Health Plan (fictional) - Medi-Cal

**Member**

Member Name: Kevin Doe

DOB: 02/17/2002

Member ID: OCH-73300195

**Referring Provider**

Facility: Seaside Community Medical Center (fictional)

**Clinical**

Primary Diagnosis: Laceration of left forearm with cellulitis

ICD-10: S51.812A, L03.114

Member is experiencing homelessness [X]

Member at risk of hospitalization/re-hospitalization [X]

Requested Start Date: 09/05/2026

Requested Days: 14

**Plan Use Only**

Status: Pending review

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# Patient face sheet

**Seaside Community Medical Center (fictional) - PATIENT FACE SHEET**

Patient: Kevin Doe
MRN: SC-5072218
Account #: 99072218
DOB: 02/17/2002
Sex: Male
Preferred Language: English
Ethnicity: Hispanic or Latino
Marital Status: Single
Address: NO FIXED ADDRESS (homeless)
Mailing: 1234 Example Ave, Santa Ana, CA 92700 (shelter mail drop)
Phone: 555-0100 (message)

**Encounter**

Admit Date/Time: 09/01/2026 04:20
Admit Type: Inpatient
Unit/Bed: 5M-509
Attending: Fumiko Nakamura, MD

**Insurance**

Primary: Orange Coast Health Plan (fictional) - Medi-Cal
Subscriber: Self

**Emergency Contact**

None listed

**Alerts**

Behavioral alert: YES (added 09/03/2026)
Security flag: YES

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## History and Physical - page 1

**HISTORY AND PHYSICAL**

Date: 09/01/2026

**CC**

Cut on L arm, now red and swollen

**HPI**

24 y/o M, unhoused, reports he fell onto a broken bottle 3 days ago. Wound not cleaned. Now erythema, swelling and purulent drainage. Pt is guarded, gives limited history. Reports he "used to take a pill for my head" (prior risperidone per outside med list), stopped ~3 months ago.

**PMH**

- Schizophrenia (per outside records, dx 2021; prior psych hospitalization x2)

**Meds PTA**

- None (prior risperidone 2 mg BID - not taking)

**Allergies**

NKDA

**Social**

Unhoused since age 19, multiple episodes. Cannabis daily, methamphetamine "sometimes" (last 1 wk ago). EtOH occasional.

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## History and Physical - page 2

**Exam**

T 100.6F HR 106 BP 132/80 RR 18 SpO2 99% RA
L dorsal forearm: 6 cm laceration with purulent drainage, surrounding erythema 8 cm, no foreign body on XR, tendons intact
Psych: guarded, internally preoccupied at times, AOx4

**Assessment/Plan**

1. Infected L forearm laceration with cellulitis - irrigation and debridement at bedside, delayed primary closure 09/02, IV cefazolin then PO
2. Schizophrenia, off meds - psych consult if behavioral concerns
3. Substance use - UDS, SW consult

Signed: Fumiko Nakamura, MD 09/01/2026 07:30

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# Nursing note - behavioral event

**NURSING NOTE - BEHAVIORAL EVENT**

Date/Time: 09/03/2026 23:15

Pt became agitated when asked to return to room after being found in another pt's room. Pt yelled profanities and threats at RN ("I'll f--- you up"), threw meal tray toward RN (missed, struck wall). Code Gray called; security to bedside 23:18.

Pt stated voices were "telling me you're poisoning me."

Charge RN and MD notified. Olanzapine 5 mg IM x1 given 23:35 per order. Pt calmed by 00:10.

1:1 sitter initiated. Behavioral alert placed in chart.

Incident report filed.

Signed: D. Achebe, RN / Charge RN P. Lyons

**NURSING NOTE 09/04/2026 14:00**

Pt verbally aggressive toward CNA during vitals, refused, security on standby. Remains on 1:1.

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## Psychiatry consult

**PSYCHIATRY CONSULTATION**

Date: 09/04/2026 10:30

Reason: agitation, aggression toward staff 09/03, hx schizophrenia off meds

**Interview**

Pt guarded, intermittently responding to internal stimuli. Endorses auditory hallucinations x weeks: male voices that "tell me to hurt people who are trying to poison me", including staff. Reports he sometimes "has to listen." Denies SI. Denies plan to harm specific person "right now." Poor insight.

MSE: disheveled, psychomotor restless, speech pressured, mood "annoyed", affect irritable, thought process tangential, + paranoid delusions, + command AH, cognition grossly intact, AOx4. Judgment/insight poor.

**Assessment**

Schizophrenia, acute exacerbation in setting of medication nonadherence and methamphetamine use. Command auditory hallucinations to harm others. Aggression toward staff 09/03 and 09/04.

**Recommendations**

- Restart risperidone 1 mg PO BID, titrate to 2 mg BID
- Olanzapine 5 mg IM/PO q6h PRN severe agitation
- Continue 1:1 sitter
- 5150 evaluation: does not meet criteria at this exact time; reassess daily; low threshold
- Recommend psychiatric stabilization before discharge to any congregate setting

Signed: G. Iqbal, MD (Psychiatry)

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## Hospitalist progress note

**HOSPITALIST PROGRESS NOTE - HD#5**

Date: 09/05/2026 07:50

S: Pt slept 4 hrs. Took risperidone last night. Refused AM labs.

O: T 98.4F HR 88 BP 128/78. Wound: sutures intact, erythema resolved to 2 cm. AOx4. Remains on 1:1.

A/P:

1. Cellulitis L forearm - improved, cephalexin 500 mg PO QID through 09/08
2. Schizophrenia exacerbation - per psych, risperidone started 09/04; continue 1:1
3. Dispo - medically stable from infection standpoint. CM pursuing recuperative care.

Signed: Fumiko Nakamura, MD

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## Vitals and labs

**VITAL SIGNS - LAST 5 SETS**

| Date/Time | Temp (F) | HR | BP | RR | SpO2 | O2 |
|---|---|---|---|---|---|---|
| 09/03 20:00 | 98.6 | 96 | 136/84 | 18 | 99% | RA |
| 09/04 08:00 | 98.2 | 92 | 130/80 | 16 | 99% | RA |
| 09/04 14:00 | -- | -- | -- | -- | -- | refused |
| 09/04 20:00 | 98.5 | 90 | 128/76 | 16 | 98% | RA |
| 09/05 07:50 | 98.4 | 88 | 128/78 | 16 | 99% | RA |

**LABS**

09/01 | WBC 15.2 H | Hgb 14.6 | Plt 260 | Cr 0.9
09/03 | WBC 9.4
09/01 UDS: + cannabinoids, + amphetamines
09/01 Wound cx: Strep pyogenes (GAS), pan-sensitive
09/01 MRSA nares PCR: Negative
09/05 SARS-CoV-2 rapid antigen: Not collected (pt refused 09/05 AM)
09/01 L forearm XR: no foreign body, no fracture

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## Medication list

**CURRENT MEDICATIONS / ANTICIPATED DISCHARGE**

Patient: Kevin Doe MRN: SC-5072218

- Cephalexin 500 mg PO four times daily through 09/08/2026
- Risperidone 1 mg PO BID (NEW 09/04/2026), titration per psychiatry
- Acetaminophen 650 mg PO q6h PRN pain

**Inpatient PRN**

- Olanzapine 5 mg IM/PO q6h PRN severe agitation (given 09/03 23:35)

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